Colon & Colorectal Cancer — FDA Approvals & Research Intelligence

Alert Window: FDA Approval Reference · Generated: 2026-03-18 19:16:30

Clinical Summary

This report covers 11 FDA-approved colorectal cancer treatments, spanning 2020–2026. 10 of 11 approvals target Stage IV (metastatic) disease. 10 approvals are biomarker-driven, requiring a specific genetic test before a patient qualifies — the dominant biomarkers are BRAF (3) · PD-1/PD-L1 (2) · KRAS (2) · VEGF (1). The approvals span targeted therapies, immunotherapies, and combination regimens for molecularly defined patient populations. Each entry in this report represents a treatment available today — not a research study — and is linked directly to its FDA approval announcement.

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Situation Summary

Activity	**11 entries** all approvals
Stage IV	**10 of 11**
Biomarker-Targeted	**10 entries** BRAF (3) · PD-1/PD-L1 (2) · KRAS (2) · VEGF (1)
Phase Distribution	**Not specified**
Breakthrough Signals	**11 entries scored ≥ 4**
Research Activity	**High Activity**

Approvals & Signals

FDA approvals and high-significance entries ranked by biomarker, therapy class, and phase signals. Higher scores indicate greater clinical significance. Always discuss with your oncologist.

FDA grants traditional approval to encorafenib for metastatic colorectal cancer with a BRAF V600E mutation

STAGE IV BRAF Score 11 PUBLISHED FEB 24, 2026

The FDA has granted traditional approval to encorafenib for the treatment of adult patients with metastatic colorectal cancer (CRC) who have a BRAF V600E mutation. This approval allows encorafenib to be used in combination with cetuximab and fluorouracil-based chemotherapy, which is significant for targeting this specific mutation. The BRAF V600E mutation is a key biomarker that can influence treatment decisions in metastatic CRC. This approval builds on the previous accelerated approval of encorafenib in 2024, addressing the need for effective therapies for this patient population.

Summary derived from FDA approval announcement · Verify current prescribing information with your oncologist.

FDA approves sotorasib with panitumumab for KRAS G12C-mutated colorectal cancer

STAGE IV KRAS Score 11 PUBLISHED JAN 16, 2025

The study reports the FDA's approval of sotorasib in combination with panitumumab for adult patients with KRAS G12C-mutated metastatic colorectal cancer who have previously undergone specific chemotherapy treatments. Sotorasib targets the KRAS G12C mutation, which is significant as this mutation is often associated with treatment resistance in colorectal cancer. The approval is contingent upon the presence of the KRAS G12C mutation, identified through an FDA-approved test. This addresses the need for effective treatment options for patients with this specific genetic alteration who have limited therapeutic alternatives.

Summary derived from FDA approval announcement · Verify current prescribing information with your oncologist.

FDA grants accelerated approval to encorafenib with cetuximab and mFOLFOX6 for metastatic colorectal cancer with a BRAF V600E mutation

STAGE IV BRAF Score 11 PUBLISHED DEC 20, 2024

The FDA has granted accelerated approval for the combination of encorafenib, cetuximab, and mFOLFOX6 for patients with metastatic colorectal cancer (mCRC) who have a BRAF V600E mutation. This combination therapy is significant as it targets the specific mutation associated with poorer outcomes in colorectal cancer. The presence of the BRAF V600E mutation, identified through an FDA-approved test, is a critical biomarker for determining eligibility for this treatment. This approval addresses the need for effective treatment options for patients with this specific genetic profile.

Summary derived from FDA approval announcement · Verify current prescribing information with your oncologist.

FDA grants accelerated approval to adagrasib with cetuximab for KRAS G12C-mutated colorectal cancer

STAGE IV KRAS Score 11 PUBLISHED JUN 21, 2024

The FDA has granted accelerated approval for the combination of adagrasib and cetuximab for adults with KRAS G12C-mutated locally advanced or metastatic colorectal cancer. Adagrasib targets the KRAS G12C mutation, which is significant as this mutation can drive cancer progression and may limit treatment options. Patients must have previously undergone treatment with fluoropyrimidine-, oxaliplatin-, and irinotecan-based chemotherapy to qualify for this therapy. This approval addresses the need for effective treatment options for patients with this specific genetic mutation in colorectal cancer.

Summary derived from FDA approval announcement · Verify current prescribing information with your oncologist.

FDA grants accelerated approval to tucatinib with trastuzumab for colorectal cancer

STAGE IV | HER2 | Score 11 | PUBLISHED JAN 19, 2023

The FDA has granted accelerated approval for the combination of tucatinib and trastuzumab for patients with RAS wild-type HER2-positive unresectable or metastatic colorectal cancer that has progressed after standard chemotherapy treatments. Tucatinib is a targeted therapy that inhibits the HER2 protein, which plays a role in the growth of certain cancer cells. This approval specifically addresses patients whose cancer has not responded to previous treatments, highlighting the importance of identifying HER2-positive tumors. This study provides a new treatment option for a specific subset of colorectal cancer patients facing limited alternatives.

Summary derived from FDA approval announcement · Verify current prescribing information with your oncologist.

FDA approves encorafenib in combination with cetuximab for metastatic colorectal cancer with a BRAF V600E mutation

STAGE IV | BRAF | Score 11 | PUBLISHED APR 8, 2020

The FDA has approved encorafenib in combination with cetuximab for the treatment of adult patients with metastatic colorectal cancer who have a BRAF V600E mutation. This combination targets the specific mutation, which is significant for patients who have not responded to prior therapies. The presence of the BRAF V600E mutation, identified through an FDA-approved test, is a critical biomarker for determining eligibility for this treatment. This approval addresses the need for effective options in a patient population with limited responses to existing therapies.

Summary derived from FDA approval announcement · Verify current prescribing information with your oncologist.

FDA approves nivolumab with ipilimumab for unresectable or metastatic MSI-H or dMMR colorectal cancer

STAGE IV | PD-1/PD-L1 | Score 7 | PUBLISHED APR 8, 2025

The study reports the FDA's approval of nivolumab in combination with ipilimumab for patients aged 12 and older with unresectable or metastatic microsatellite instability-high (MSI-H) or mismatch repair deficient (dMMR) colorectal cancer. This combination therapy is significant as it targets specific genetic characteristics of the cancer, potentially improving treatment outcomes for this patient population. The relevant biomarkers for this approval are MSI-H and dMMR, which indicate a specific type of colorectal cancer that may respond to this treatment. This approval addresses the need for effective therapies in patients whose cancer has progressed after standard treatments.

Summary derived from FDA approval announcement · Verify current prescribing information with your oncologist.

FDA approves fruquintinib in refractory metastatic colorectal cancer

STAGE IV | GENERAL | Score 7 | PUBLISHED NOV 8, 2023

The FDA has approved fruquintinib (Fruzaqla) for adult patients with refractory metastatic colorectal cancer (mCRC). Fruquintinib is a targeted therapy that inhibits angiogenesis, which is the formation of new blood vessels that tumors need to grow. This approval is particularly relevant for patients who have already undergone treatment with fluoropyrimidine-, oxaliplatin-, and irinotecan-based chemotherapy, as well as anti-VEGF therapy, and, if applicable, anti-EGFR therapy for those with RAS wild-type tumors. This study addresses the need for effective treatment options in patients with advanced disease who have exhausted standard therapies.

Summary derived from FDA approval announcement · Verify current prescribing information with your oncologist.

FDA approves trifluridine and tipiracil with bevacizumab for previously treated metastatic colorectal cancer

STAGE IV VEGF Score 7 PUBLISHED AUG 2, 2023

The FDA has approved the combination of trifluridine and tipiracil (LONSURF) with bevacizumab for patients with previously treated metastatic colorectal cancer (mCRC). This combination is significant as it builds on the previously established use of LONSURF as a single agent, enhancing treatment options for patients who have already undergone multiple lines of chemotherapy. The relevant patient characteristic for this treatment is that it is intended for those who have been treated with fluoropyrimidine-, oxaliplatin-, and irinotecan-based therapies, as well as specific biological therapies based on their RAS status. This approval addresses the need for effective treatment alternatives in a patient population with limited options.

Summary derived from FDA approval announcement · Verify current prescribing information with your oncologist.

FDA approves pembrolizumab for first-line treatment of MSI-H/dMMR colorectal cancer

STAGE IV PD-1/PD-L1 Score 7 PUBLISHED JUN 29, 2020

The FDA has approved pembrolizumab (KEYTRUDA) for the first-line treatment of patients with unresectable or metastatic colorectal cancer that is microsatellite instability-high (MSI-H) or mismatch repair deficient (dMMR). Pembrolizumab is an immunotherapy that works by targeting the PD-1 pathway, which plays a role in the immune system's ability to fight cancer. This approval specifically addresses patients with MSI-H or dMMR characteristics, which are important biomarkers for determining eligibility for this treatment. This study provides a new treatment option for a specific subset of colorectal cancer patients who may benefit from immunotherapy.

Summary derived from FDA approval announcement · Verify current prescribing information with your oncologist.

FDA approves new dosing regimen for cetuximab

ALL STAGES EGFR Score 5 PUBLISHED APR 6, 2021

The FDA has approved a new dosing regimen for cetuximab, specifically for patients with K-Ras wild-type, EGFR-expressing colorectal cancer or squamous cell carcinoma of the head and neck. This new regimen involves administering cetuximab at a dosage of 500 mg/m2 as a 120-minute intravenous infusion every two weeks. The relevant patient characteristic for this treatment is the presence of K-Ras wild-type and EGFR expression. This approval addresses the need for optimized dosing strategies in the treatment of these specific cancer types.

Summary derived from FDA approval announcement · Verify current prescribing information with your oncologist.

Event Timeline

FDA grants traditional approval to encorafenib for metastatic colorectal cancer with a BRAF V600E mutation

HIGH STAGE IV BRAF FDA APPROVAL VERIFIED PUBLISHED FEB 24, 2026 11

fda.gov

This study evaluates the traditional approval granted by the FDA to encorafenib for metastatic colorectal cancer with a BRAF V600E mutation.

FDA approves nivolumab with ipilimumab for unresectable or metastatic MSI-H or dMMR colorectal cancer

HIGH STAGE IV PD-1/PD-L1 FDA APPROVAL VERIFIED PUBLISHED APR 8, 2025 7

fda.gov

This study evaluates the FDA approval of nivolumab with ipilimumab for unresectable or metastatic MSI-H or dMMR colorectal cancer.

FDA approves sotorasib with panitumumab for KRAS G12C-mutated colorectal cancer

HIGH STAGE IV KRAS FDA APPROVAL VERIFIED PUBLISHED JAN 16, 2025 11

fda.gov

This study evaluates the FDA approval of sotorasib with panitumumab for KRAS G12C-mutated colorectal cancer.

FDA grants accelerated approval to encorafenib with cetuximab and mFOLFOX6 for metastatic colorectal cancer with a BRAF V600E mutation

HIGH STAGE IV BRAF FDA APPROVAL VERIFIED PUBLISHED DEC 20, 2024 11

fda.gov

This study evaluates the FDA granting accelerated approval to encorafenib with cetuximab and mFOLFOX6 for metastatic colorectal cancer with a BRAF V600E mutation.

FDA grants accelerated approval to adagrasib with cetuximab for KRAS G12C-mutated colorectal cancer

HIGH | STAGE IV | KRAS | FDA APPROVAL | VERIFIED | PUBLISHED JUN 21, 2024 | 11

fda.gov

This study evaluates the FDA granting accelerated approval to adagrasib with cetuximab for KRAS G12C-mutated colorectal cancer.

FDA approves fruquintinib in refractory metastatic colorectal cancer

HIGH | STAGE IV | GENERAL | FDA APPROVAL | VERIFIED | PUBLISHED NOV 8, 2023 | 7

fda.gov

This study evaluates the FDA approval of fruquintinib in refractory metastatic colorectal cancer.

FDA approves trifluridine and tipiracil with bevacizumab for previously treated metastatic colorectal cancer

HIGH | STAGE IV | VEGF | FDA APPROVAL | VERIFIED | PUBLISHED AUG 2, 2023 | 7

fda.gov

This study evaluates the FDA approval of trifluridine and tipiracil with bevacizumab for previously treated metastatic colorectal cancer.

FDA grants accelerated approval to tucatinib with trastuzumab for colorectal cancer

HIGH | STAGE IV | HER2 | FDA APPROVAL | VERIFIED | PUBLISHED JAN 19, 2023 | 11

fda.gov

This study evaluates the FDA granting accelerated approval to tucatinib with trastuzumab for colorectal cancer.

FDA approves new dosing regimen for cetuximab

HIGH | ALL STAGES | EGFR | FDA APPROVAL | VERIFIED | PUBLISHED APR 6, 2021 | 5

fda.gov

This study evaluates the FDA approval of a new dosing regimen for cetuximab.

FDA approves pembrolizumab for first-line treatment of MSI-H/dMMR colorectal cancer

HIGH | STAGE IV | PD-1/PD-L1 | FDA APPROVAL | VERIFIED | PUBLISHED JUN 29, 2020 | 7

fda.gov

This study evaluates the FDA approval of pembrolizumab for first-line treatment of MSI-H/dMMR colorectal cancer.

FDA approves encorafenib in combination with cetuximab for metastatic colorectal cancer with a BRAF V600E mutation

HIGH | STAGE IV | BRAF | FDA APPROVAL | VERIFIED | PUBLISHED APR 8, 2020 | 11

fda.gov

This study evaluates the FDA approval of encorafenib in combination with cetuximab for metastatic colorectal cancer with a BRAF V600E mutation.

Appendix A — Patient-Friendly Summary

Plain-language overview for patients, caregivers, and family members

What's happening

The FDA has approved the following targeted treatments for colorectal cancer, each requiring specific biomarker testing to determine eligibility: encorafenib (approved 2026) for patients with BRAF mutations; sotorasib with panitumumab (approved 2025) for patients with KRAS mutations; encorafenib with cetuximab and mFOLFOX6 (approved 2024) for patients with BRAF mutations; adagrasib with cetuximab (approved 2024) for patients with KRAS mutations; tucatinib with trastuzumab (approved 2023) for patients with HER2 mutations. Each of these treatments is currently available — not experimental.

What this means for you

For patients, the most important step is biomarker testing at diagnosis. Ask your oncologist to test for BRAF V600E, KRAS G12C, MSI-H/dMMR, and HER2 — these results determine which of the approved treatments above may apply to you. Bring this report to your next appointment and ask specifically: 'Have I been tested for all the biomarkers that have an FDA-approved treatment?' If you have BRAF V600E, ask about encorafenib. If you have MSI-H or dMMR, ask about pembrolizumab or nivolumab with ipilimumab. If you have KRAS G12C, ask about sotorasib with panitumumab or adagrasib with cetuximab.

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This is not medical advice. Always speak with your oncologist before making any treatment decisions.

Appendix B — Colon & Colorectal Cancer Treatment Reference

Incidence & Trends

~150,000 new US cases/year · ~52,000 deaths/year · 4th most common US cancer

▲ Early-Onset Alert: CRC rates in adults under 50 have risen ~2% per year since the 1990s · Adults under 55 now account for ~20% of all new CRC diagnoses · Cause under active investigation (diet, microbiome, obesity, sedentary lifestyle)

Current Screening Guidelines (2025)

USPSTF / ACS / ACG — Average Risk: Begin screening at age 45 (lowered from 50 in 2021)

Colonoscopy every 10 years · FIT (fecal immunochemical test) annually · Cologuard every 1-3 years · CT colonography every 5 years

High Risk (family history / Lynch syndrome / IBD): Begin at age 40 or 10 years before youngest affected relative — whichever comes first · Colonoscopy every 1-5 years depending on risk

Symptoms at any age: Rectal bleeding, change in bowel habits >4 weeks, unexplained weight loss, iron deficiency anemia → immediate evaluation regardless of age

Key Biomarkers — Test All mCRC at Diagnosis

RAS (KRAS/NRAS) · BRAF V600E · MSI/MMR status · HER2 · NTRK fusion · TMB

Stage IV Targeted Options

KRAS G12C (~3-4%) → sotorasib + cetuximab · adagrasib + cetuximab

BRAF V600E (~8-10%) → encorafenib + cetuximab ± binimetinib

MSI-H/dMMR (~5%) → pembrolizumab (1st line preferred) · nivolumab ± ipilimumab

HER2 amplified (~2-3%) → trastuzumab + pertuzumab · trastuzumab deruxtecan

RAS/BRAF WT → FOLFOX or FOLFIRI + cetuximab or panitumumab (left-sided preferred)

1st Line mCRC Standard of Care

FOLFOX or FOLFIRI ± bevacizumab (RAS mutant / right-sided)

FOLFOX or FOLFIRI + cetuximab or panitumumab (RAS/BRAF WT, left-sided)

Pembrolizumab monotherapy (MSI-H/dMMR)

Key Resources

ClinicalTrials.gov · NCCN Guidelines (nccn.org) · Colorectal Cancer Alliance (ccalliance.org) · Fight Colorectal Cancer (fightcrc.org) · ACS (cancer.org)

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